Pathways to Recovery for Pregnant & Parenting Women (PPW) in Northern Arizona: A Needs Assessment

"The system works when the people do; but it should work even when they're exhausted."

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Introduction

Local Context:
Compassionate but structurally fragile ecosystem for pregnant and parenting women (PPW) living with a substance use disorder (SUD) in Northern Arizona.

Purpose:
Analyze: Identify strengths, gaps, and system-level opportunities for PPW care in Northern Arizona.

Investigate: Understand how rural, behavioral, and public health systems serve PPW with SUD.

Inform: Develop coordinated, evidence-informed, and culturally responsive approaches to improve regional outcomes.

Methods

Focus: Structural gaps, promising practices, and actionable systems of change analyzed through rapid qualitative analysis and cross-case synthesis of 5 analytic domains.
Data Collection: Participants (n=22) from clinical, public health, justice, tribal leadership, and community members participated in 17 semi-structured 45-60-minute interviews (Zoom, auto-recorded and -transcribed) from May - October 2025 across these domains 1) Identifying and engaging PPW with SUD, 2) Current protocols and services, 3) Barriers to care, 4) Facilitators and promising practices, and 5) System-level reflections.

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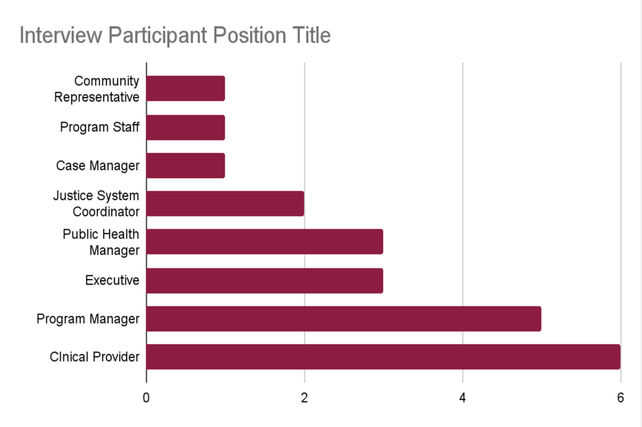
Results

Overview of Emergent Themes: Across the 17 interviews, participants expressed consensus on system needs and barriers, alongside alignment on promising approaches that center relationship- based, trauma-informed, and culturally responsive care.

Domain	Most Frequent Themes	Illustrative Highlights
Identifying & Engaging PPW with SUD	Relationship-based engagement; multiple entry points; fear and stigma limit disclosure	Identification relies on trust and peer outreach rather than formal screening protocols.
Current Protocols & Services	Comprehensive but fragmented service landscape; strong local innovation	Effective dyadic care, MAT integration, and perinatal support exist but vary by geography and funding.
Barriers to Care	Housing scarcity; transportation and childcare gaps; workforce shortages; fragmented coordination	Housing instability consistently described as the greatest barrier to recovery and family reunification.
Facilitators & Promising Practices	Peer integration; cross-sector collaboration; reflective supervision; culturally grounded healing	Peer and paraprofessional roles enhance trust and continuity; mom/baby programs are strong facilitators.
System-Level Reflections	Need for sustainable financing, data sharing, and Indigenous-led reform	Stakeholders called for bundled maternal behavioral-health payments and centralized navigation systems.



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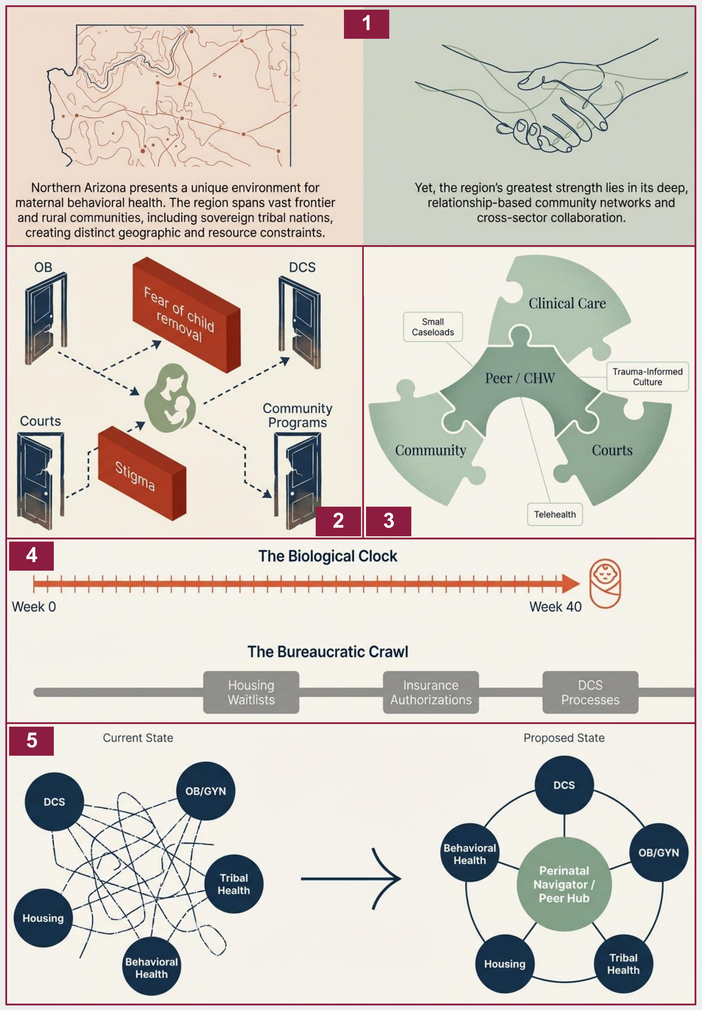
1. **A Compassionate but Fragile Ecosystem:** Currently, this system relies entirely on the exhaustion and dedication of its people, operating without the formal infrastructure necessary to sustain it.
2. **Identification Relies on Trust. Not Protocols:** Engagement happens through multiple entry points, but screening remains inconsistent. Substance use is frequently hid due to fear of child removal and systemic stigma. Engagement depends heavily on early connection, nonjudgmental tone, and the continuity of peer outreach.
3. **Peers and Paraprofessionals as System Connectors:** Trust is the primary currency in a fragmented landscape. Community Health Workers (CHWs), Doulas, and Peer Support Specialists (PSS) with lived experience partially compensate for clinician shortages, reduce stigma, and sustain engagement via relationship-based care. This transforms punitive systemic structures into recovery ecosystems.
4. **The Temporal Mismatch:** Pregnancy creates a narrow, time-sensitive window for intervention and motivation. Bureaucratic processes do not operate on this biological clock.
5. **From Fragmentation to Integration:** Replacing parallel, disjointed systems with a single, coordinated entry and navigation process.



Discussion

Cross-Cutting Insights: Fragmentation is the core system problem. Relationships, not infrastructure, currently hold the system together. Housing, peers, and cultural responsiveness drive engagement more than any single clinical service.

Sustainable change = aligning people, policy, and payment.

Policy & Payer Implications:

- Payment parity for peers, CHWs, doulas, and audio-only visits.
- Payer pilots should tie maternal outcomes to avoided downstream costs (ED, NICU, foster care).
- Pilot evidence may justify long-term Medicaid and state investment.

Recommendations

Immediate

- Pregnant PPW Priority Pathway: Standardize screening + 48–72-hour warm handoffs.
- Assign perinatal navigators/peers as single contact points.
- Develop HIPAA/data-sharing primers for cross-agency coordination.

Short-Term

- Stabilize paraprofessional roles through pilot Medicaid reimbursement.
- Regional resource directory for housing, MAT, and supports.
- Expand training (PSI, MI, reflective supervision) across agencies.

Long-Term

- Maternal Behavioral Health Bundled Payment (pregnancy → 12 months postpartum).
- Develop mother-and-baby residential capacity.
- Shared data platform / intake hub linking DCS, courts, OBs, and BH.
- Embed Indigenous leadership and ICWA-informed courts.